DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

MEMORANDUM FOR RECORD

To: Clinton Pope, Ms. Ward Lund, and Relevant Policy Owners
From: Kevin D. Turnage, MSN Ed., RN, NE-BC, NHDP-BC, NPD-BC
Date: May 22, 2025
Subject: Revised Review – Safety Event Reporting Policy, Sentinel Event Framework, and Risk Analysis

Purpose

This memorandum provides a revised analysis of the **Safety Event Reporting Policy**, incorporating both the content of the policy itself and its associated attachments. It also includes a legal and regulatory perspective on the implications of relying on supplementary documents that are not formally incorporated into the main policy.

Context of Initial Review

My original assessment was limited to the core policy narrative and did not account for supporting attachments. These attachments, now reviewed, include:

- RCA Flowchart
- Safety Event Decision-Making Guide
- Safety Event Reporting Flowchart
- I-STAR Handler Instructions
- FMEA Template

At that time, I identified gaps in how the policy addressed sentinel events, root cause analysis (RCA) requirements, and leadership oversight responsibilities.

Updated Findings

After reviewing the attachments in conjunction with the policy, the following improvements are evident:

1. Inclusion of Sentinel Events within "Reportable Events"
The attachments clearly state that reportable events include those meeting The Joint Commission's definition of sentinel events (SE-3 to SE-6).

2. Well-Structured RCA and CAP Processes
The attachments outline a defined process for RCA initiation, RCA charter development, team formation, timeline tracking, and executive briefing of final corrective actions.

3. Clear Executive Oversight Pathways
Flowcharts show structured involvement of the CQO, CEO, Executive Sponsor, and Quality Council for all qualifying events.

Remaining Gaps and Legal Risks

Despite these strengths, key issues persist within the policy structure and content:

1. Absence of a Direct Sentinel Event Definition in the Policy Text
The main policy does not explicitly define "sentinel event," though it is included in the definition of "reportable event" in attachments.

Recommendation: Add a definition to the Definitions section:
Sentinel Event (SE): A patient safety event resulting in death, permanent harm, or severe temporary harm, signaling the need for immediate investigation and response, as defined by The Joint Commission (SE-3 to SE-6).

2. Unclear Link Between "Reportable" and "Sentinel" Events in the Policy
The connection is implied but not stated in the body of the policy.

Recommendation: Add to the Scope or Policy Statement section:
For the purposes of accreditation and regulatory compliance, all events classified as Reportable Events include those defined by The Joint Commission as Sentinel Events.

3. Attachments Are Referenced, But Not Formally Incorporated into Policy

The policy includes an "Attachments" section listing several documents, and the narrative includes informal references such as "See attached RCA Flowchart." However, there is no language stating that these attachments are formally **incorporated by reference** or that they carry **binding policy authority**.

Legal and Regulatory Risk:

- **Survey Deficiency Risk:** Regulatory surveyors (CMS, TJC) typically require that essential procedures appear in or are explicitly incorporated into the main policy.
- **Enforceability Risk:** Attachments may be treated as non-binding unless formally incorporated.
- **Employment Law Risk:** In disciplinary or grievance contexts, unclear policies may fail to provide adequate notice or defensible expectations.
- **Legal Exposure:** In litigation or adverse event inquiries, reliance on non-incorporated flowcharts could undermine the organization's defense.

Recommendation: Amend the policy to include the following or similar language:
The attachments listed herein are incorporated by reference and constitute enforceable

components of this policy. All staff are required to adhere to the procedures, responsibilities, and timelines detailed within the attachments.

4. Staff-Facing Clarity Still Limited
The policy does not contain a quick-reference tool or appendix listing examples of sentinel events.

Recommendation: Include an appendix with common examples, such as:

- Suicide within 7 days of discharge
- Maternal death related to childbirth
- Surgery on the wrong body part or patient
- Retained surgical items

Conclusion

While the supporting materials represent substantial operational improvements, the lack of integration into the main policy creates material risks. The current structure leaves the organization vulnerable to misinterpretation by regulators, staff, and legal reviewers. Policy revisions should address this by:

- Adding a clear definition of sentinel events.
- Stating explicitly that reportable events include sentinel events.
- Incorporating all attachments by formal reference.
- Including key procedural content directly in the policy text.
- Providing a sentinel event reference guide for staff clarity.

These changes will enhance regulatory readiness, legal defensibility, and internal consistency.

I remain available to assist with policy editing, formatting, and implementation.

Respectfully submitted,

Digitally signed by Kevin Turnage
DN: cn=Kevin Turnage, o=Phoenix Indian Medical Center, ou=HHS, email=kevin.turnage@ihs.gov, c=US
Date: 2025.05.22 15:42:24 -07'00'

Kevin D. Turnage, MSN Ed., RN, NE-BC, NHDP-BC, NPD-BC
Policy and Procedure Nurse Consultant
Phoenix Indian Medical Center